SmartRx Digital Prescription

Official Medical Prescription Record

Doctor	Dr. Mohiuddin Mukim
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Medicine Name	Dosage	Important Info
Amoclav (Amoxicillin + Clavulanic Acid)	1-0-1	Indication: Co-amoxiclav is indicated for short-term treatment of bacterial infections at the following sites: Upper respiratory tract infections (including ENT) e.g.tonsillitis,sinusitis,otitis media. Lower respiratory tract infections e.g.acute and chronic bronchitis, lobar and bronchopneumonia. ... Read more Co-amoxiclav is indicated for short-term treatment of bacterial infections at the following sites: Upper respiratory tract infections (including ENT) e.g.tonsillitis,sinusitis,otitis media. Lower respiratory tract infections e.g.acute and chronic bronchitis, lobar and bronchopneumonia. Genito-urinary tract infections e.g.cystitis,urethritis,pyelonephritis. Skin and soft tissue infections. Bone and joint infections e.g.osteomyelitis. Other infections e.g.septic abortion,puerperal sepsis,intra-abdominal sepsis etc. Pharmacology: Pharmacodynamic properties: Co-amoxiclav is an antibacterial combination consisting of the antibiotic Amoxicillin and the (3-lactamase inhibitor Clavulanic Acid. Amoxicillin has a broad spectrum of bactericidal activity against many Gram-positive & Gram-negative microorganisms but it is susceptible to degradation by (3-lactamases and therefore the spectrum of activity does not include microorganisms, which produce these enzymes. Clavulanic acid possesses the ability to inactivate a wide range of beta-lactamase enzymes commonly found in microorganisms resistant to penicillins and cephalosporins. Thus Clavulanic acid in this preparation protects Amoxicillin from degradation by (3-lactamase enzymes and effectively extends the antibiotic spectrum to embrace a wide range of microorganisms. Pharmacokinetic properties: The pharmacokinetics of the two components of Co-amoxiclav is closely matched. Peak serum levels of both occur about one hour after oral administration. Absorption of Co-amoxiclav is optimized at the start of a meal. Both clavulanate and Amoxicillin have low levels of serum binding; about 70% remains free in the serum. Doubling the dosage of Co-amoxiclav approximately doubles the serum levels achieved. Dosage: Adults and children over 12 years: Tablet: The usual adult dose is one 625 mg Tablet every 12 hours or one 375 mg Tablet every 8 hours. For more severe infections and infections of the respiratory tract, the dose should be one 1 gm Tablet every 12 hours or one 625 mg Tablet every 8 hours. Suspension: Children 6-12 years: 2 teaspoonful every 8 hours. Children 1-6years: 1 teaspoonful every 8 hours. Children below 1 year: 25 mg/kg/day in divided doses every 8 hours, for example a 7.5 kg child would require 2 ml suspension t.i.d, Treatment should not be extended beyond 14 days without review. Forte suspension: The usual recommended daily dosage: 25/3.6 mg/kg/day in mild to moderate infections (upper respiratory tract infections e.g. recurrent tonsilitis, lower respiratory infections, and skin and soft tissue infections) For serious infections: 45/6.4 mg/kg/day for the treatment of more serious infections (upper respiratory tract infections, e.g. otitis media and sinusitis, lower respiratory infections e.g. bronchopneumonia, and urinary tract infections). Children of 2 to 12 years: Mild to moderate infections: 25/3.6 mg/kg/day (Suspension) 2-6 years (13-21 kg) 2.5 ml suspension b.i.d 7-12years (22-40kg) 5 ml suspension b.i.d Serious infections: 45/6.4 mg/kg/day (Forte Suspension) 2-6 years (13-21 kg) 5 ml suspension b.i.d 7-12 years (22-40 kg) 10 ml suspension b.i.d IV Injection Adults- Usually, 1.2 gm every 8 hours Increased in more serious infections to 1.2 gm every 6 hours For surgical prophylaxis: The usual dose is 1.2 gm at induction, for high risk procedures (eg. colorectal surgery) up to 2-3 gm may be given every 8 hours. Children- 0 to 3 months: 30 mg/kg every 8 hours. (every 12 hours in the perinatal period and in premature infants. 3 months to 12 years: Usually 30 mg/kg every 8 hours increased in more serious infection to 30 mg/kg every 6 hours. Side Effects: Side effects, as with Amoxicillin, are uncommon and mainly of a mild and transitory nature. Diarrhoea, pseudomembranous colitis, indigestion, nausea, vomiting and candidiasis have been reported, if gastrointestinal side effects occur with oral therapy, that may be reduced by taking Co-amoxiclav at the start of meals. Hepatitis and cholestatic jaundice have been reported rarely but are usually reversible. Urticarial and erythematous rashes sometimes occur. Rarely erythema multiforme, Stevens-Johnson Syndrome and exfoliative dermatitis have been reported. In common with other beta-lactam antibiotics, angioedema and anaphylaxis have been reported. Contraindications: History of Penicillin hypersensitivity. Attention should be paid to possible cross-sensitivity with other beta-lactam antibiotics e.g. cephalosporins. Also contraindicated for patients with a previous history of Co-amoxiclav or Penicillin-associated cholestatic jaundice. Pregnancy & Lactation: Animal studies with orally and parenterally administered Co-amoxiclav have shown no

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		teratogenic effect. The drug has been used orally in human pregnancy in a limited number of cases with no untoward effect; however, the use of Co-amoxiclav in pregnancy is not recommended unless considered essential by the physician. During lactation, trace quantities of Amoxicillin can be detected in breast milk. Storage: N/A
Ezitor (Atorvastatin + Ezetimibe)	1-1-1	Indication: Therapy with lipid-altering agents should be only one component of multiple risk factor intervention in individuals at significantly increased risk for atherosclerotic vascular disease due to hypercholesterolemia. Drug therapy is indicated as an adjunct to diet when the response to a diet restricted ... Read more Therapy with lipid-altering agents should be only one component of multiple risk factor intervention in individuals at significantly increased risk for atherosclerotic vascular disease due to hypercholesterolemia. Drug therapy is indicated as an adjunct to diet when the response to a diet restricted in saturated fat and cholesterol and other nonpharmacologic measures alone has been inadequate. Primary Hyperlipidemia: This tablet is indicated for the reduction of elevated total cholesterol (total-C), low density lipoprotein cholesterol (LDL-C), apolipoprotein B (Apo B), triglycerides (TG), and non-high-density lipoprotein cholesterol (non-HDL-C), and to increase high-density lipoprotein cholesterol (HDL-C) in patients with primary (heterozygous familial and non-familial) hyperlipidemia or mixed hyperlipidemia. Homozygous Familial Hypercholesterolemia (HoFH): This tablet is indicated for the reduction of elevated total-C and LDL-C in patients with homozygous familial hypercholesterolemia, as an adjunct to other lipid lowering treatments (e.g., LDL apheresis) or if such treatments are unavailable. Pharmacology: Atorvastatin: In animal models, atorvastatin lowers plasma cholesterol and lipoprotein levels by inhibiting HMG-CoA reductase and cholesterol synthesis in the liver and by increasing the number of hepatic LDL receptors on the cell surface to enhance uptake and catabolism of LDL; atorvastatin also reduces LDL production and the number of LDL particles. Ezetimibe: Ezetimibe reduces blood cholesterol by inhibiting the absorption of cholesterol by the small intestine. The molecular target of ezetimibe has been shown to be the sterol transporter, Niemann-Pick C1-Like 1 (NPC1L1), which is involved in the intestinal uptake of cholesterol and phytosterols. In a 2-week clinical study in 18 hypercholesterolemic patients, ezetimibe inhibited intestinal cholesterol absorption by 54%, compared with placebo. Ezetimibe had no clinically meaningful effect on the plasma concentrations of the fat-soluble vitamins A, D, and E and did not impair adrenocortical steroid hormone production. Ezetimibe does not inhibit cholesterol synthesis in the liver or increase bile acid excretion. Ezetimibe localizes at the brush border of the small intestine and inhibits the absorption of cholesterol, leading to a decrease in the delivery of intestinal cholesterol to the liver. This causes a reduction of hepatic cholesterol stores and an increase in clearance of cholesterol from the blood; this distinct mechanism is complementary to that of statins. Dosage: The dosage range is 10/10 mg/day through 80/10 mg/day. Recommended starting dose is 10/10 mg/day or 20/10 mg/day. Recommended starting dose is 40/10 mg/day for patients requiring a greater than 55% reduction in LDL-C. Dosing of this tablet should occur either greater than or equal to 2 hours before or greater than or equal to 4 hours after administration of a bile acid sequestrant. Side Effects: N/A Contraindications: Active liver disease or unexplained persistent elevations of hepatic transaminase levels. Hypersensitivity to any component of this tablet. Women who are pregnant or may become pregnant. Nursing mothers. Pregnancy & Lactation: Pregnancy Category X. This tablet is contraindicated in women who are or may become pregnant. Serum cholesterol and triglycerides increase during normal pregnancy. Lipid-lowering drugs offer no benefit during pregnancy, because cholesterol and cholesterol derivatives are needed for normal fetal development. Atherosclerosis is a chronic process, and discontinuation of lipid-lowering drugs during pregnancy should have little impact on long-term outcomes of primary hypercholesterolemia therapy. It is not known whether atorvastatin is excreted in human milk, but a small amount of another drug in this class does pass into breast milk. Storage: N/A

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